

DentaScale

THE PLAYBOOK

The Lead-to-Chair System

How to turn leads into booked, show-up patients — the exact steps your front desk takes between a new inquiry and a patient in the chair.

START HERE

Leads are only half the job

Getting the lead is the easy part. This is the other half.

Marketing fills your pipeline with people who raised their hand. But a lead is not a patient — and most practices quietly lose the majority of theirs to slow responses, weak phone handling, and no follow-up. The **Lead-to-Chair System** is the simple, repeatable process that closes that gap.

It's built for your front desk to use every single day. Keep it by the phone, customize the **[bracketed placeholders]** for your practice, and run the same play on every new lead.

THE ONE RULE THAT BEATS EVERYTHING

Speed. The odds of reaching and booking a lead drop sharply by the minute. Aim to respond to every new lead within **5 minutes** — a fast, human reply will out-convert a perfect-but-late one every time.

The five components

- 1 **The First Call Script** — turn a curious caller into a confirmed appointment.
- 2 **Follow-up Templates** — copy-paste texts & emails for leads who don't book on the first touch.
- 3 **The Follow-up Cadence** — exactly when and how to reach out, day by day.
- 4 **Objection Handlers** — calm, proven responses to the things leads say.
- 5 **No-Show Prevention Checklist** — keep booked patients showing up.

1 The First Call Script

A word-for-word framework to turn a new lead into a booked appointment — on the first call.

1 Open quickly & warmly — within 5 minutes of the lead coming in

"Hi [Name], this is [Your Name] from [Practice Name]. I saw you just reached out about [treatment / offer] — did I catch you at an okay time?"

2 Understand why they reached out

"Great — what made you start looking into [treatment] now?" · "Have you been to a [orthodontist / dentist] for this before?"

3 Reassure & bridge to value

"That's exactly the kind of thing we help with every day. The best next step is a quick [consultation / exam] so [Dr. Name] can take a look and give you a clear plan — no pressure to decide anything on the spot."

4 Book it — assume the appointment, offer two times

"I've actually got two openings this week — [Day] at [time] or [Day] at [time]. Which one works better for you?"

5 Lock it in

"Perfect, I've got you down for [day/time]. What's the best mobile number and email for your confirmation?"

6 Set expectations before you hang up

"You'll get a text confirmation right now and a reminder the day before. If anything comes up, just reply to that text and we'll move it — easy."

QUICK TIPS

Smile while you talk — it carries. Always offer **two specific times** rather than asking "when works for you?". Don't oversell or quote full pricing on the phone; the goal of the call is the appointment, not the close.

2 Follow-up Templates

Most leads won't pick up the first time. These get them back. Copy, paste, fill the brackets, send.

TEXT No answer — send immediately after the first call

within 5 min

Hi [Name], it's [Your Name] from [Practice Name] — just tried calling about your [treatment] inquiry. Happy to text if that's easier! When's a good time to reach you?

EMAIL Same-day value email

Day 1

Subject: Your [treatment] question — [Practice Name]

Hi [Name],
Thanks for reaching out about [treatment]. The easiest next step is a quick [consultation] with [Dr. Name] so you get a clear plan and pricing for your situation. Reply with a day that works and I'll hold a spot for you. Looking forward to meeting you!
— [Your Name], [Practice Name]

TEXT Friendly nudge

Day 3

Hi [Name], still happy to get you in for your [treatment] consult this week — I have [Day] at [time] open. Want me to save it for you?

TEXT Final touch — leave the door open

Day 7

Hi [Name], I'll stop filling your inbox 😊 If now's not the right time for [treatment], no problem at all — just reply here whenever you're ready and we'll take care of you.

MAKE IT YOURS

Keep these warm and human, never robotic. Use the lead's **first name** and the **specific treatment** they asked about every time — it's the difference between "a marketing text" and "someone who remembers me."

3 The Follow-up Cadence

When to reach out, on which channel, and why — so no lead slips through the cracks.

WHEN	CHANNEL	GOAL
Within 5 min	Call	Reach them live and book — run the First Call Script.
+5 min (no answer)	Text	Re-engage instantly while you're top of mind.
Same day	Email	Give them the info and an easy way to book.
Day 2	Call	Second live attempt, ideally a different time of day.
Day 3	Text	Friendly nudge with a specific open time.
Day 5	Call	Third and final call attempt.
Day 7	Text	Final touch — leave the door open.

RULE OF THUMB

Plan on **6–8 touches across phone, text, and email over about two weeks** before you mark a lead inactive. Most bookings happen after the 2nd–4th touch — the practices that win are simply the ones that don't stop at one.

Vary the time of day you reach out (morning vs. lunch vs. early evening) so you're not always hitting the same busy moment. Once a lead books, they exit the cadence and move straight to the No-Show Prevention checklist.

4 Objection Handlers

Calm, proven responses to what leads actually say. The goal is never to pressure — it's to remove the blocker and keep the next step easy.

"I need to think about it."

"Totally fair — it's a real decision." Most people just want to know one or two things first. What's the main thing on your mind — the cost, the timing, or how it works? Let's get that answered, and I'll hold a no-obligation spot you can always move.

"How much does it cost?"

"Great question — and it really depends on your specific situation," which is exactly why the consult is worth it. [Dr. Name] will look at what you need and give you exact pricing and any options to make it affordable. The consult itself is [free / \$X].

"Do you take my insurance?"

"Let's find out — what plan do you have?" Even if we're out of network, most patients are surprised how affordable it is, and we'll go over every option at your visit. Want me to get you scheduled so we can check it for you?

"I'm just looking right now."

"Makes sense — no rush at all." The consult is the perfect 'just looking' step: you get a clear plan and pricing with zero obligation, so whenever you are ready, you already know exactly what to expect. Shall I hold a spot?

"I need to check with my spouse."

"Of course — bring them along!" A lot of patients do. I'll book a time that works for you both so you can hear everything together and make the decision as a team.

5 No-Show Prevention Checklist

A booked appointment isn't revenue until they're in the chair. Run this on every booking.

☐ **Send a text confirmation immediately**

The moment they book, before you hang up. It makes the appointment feel real and gives them your number.

☐ **Reminder 1 — 24 hours before**

A friendly text with the date, time, and a one-tap way to confirm or reschedule.

☐ **Reminder 2 — the morning of**

Include the address, a parking/entrance tip, and what to bring. Remove every reason to be late or lost.

☐ **Personalize every message**

Use the patient's first name and the provider's name. People keep appointments with people, not with systems.

☐ **Make rescheduling effortless**

"Reply R to reschedule." A patient who reschedules is saved; a patient who ghosts is lost.

☐ **Fill cancellations the same day**

When someone cancels, immediately offer the slot to your next lead or a short-notice list.

☐ **Track your no-show rate monthly**

What gets measured gets managed. A rising rate is an early warning to tighten this checklist.

THAT'S THE SYSTEM

Run it on every lead

Consistency beats perfection. The practices that win are the ones that do this the same way, every time.

1. **The First Call Script** — book the appointment on the first call.
2. **Follow-up Templates** — re-engage the leads who don't answer.
3. **The Follow-up Cadence** — 6–8 touches over two weeks.
4. **Objection Handlers** — remove the blocker, keep the next step easy.
5. **No-Show Prevention** — turn bookings into patients in the chair.

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